PART I

Foundations and Measurement

The purposes of assessment, the logic of clinical inference, interviewing, and the psychometric basis of score interpretation.

Chapter 1 · Chapter 2 · Chapter 3 · Chapter 4

CHAPTER ONE

Foundations of Advanced Psychological Assessment and Diagnosis

Learning Objectives

After working through this chapter, the reader should be able to: - explain the purposes that psychological assessment serves in counselling psychology, and distinguish assessment from testing; - describe assessment as a sequence of decisions in which each step constrains the next; - translate a referral question into an assessment plan with proportionate methods and defensible limits; - reason with base rates, competing hypotheses and known sources of error when interpreting findings; - distinguish diagnosis, case formulation and treatment planning as related but non-identical products; - identify the technical, cultural and ethical limits that qualify any assessment conclusion.

Key terms: assessment versus testing · referral question · competing hypotheses · case formulation · differential diagnosis · base rate · functional impairment · evidence-based assessment · multimethod assessment · decision threshold · interpretive limit

Introduction

Advanced assessment is not a longer version of routine testing. What changes at doctoral level is the *quality of the inference* the practitioner is willing to defend. A student who can administer and score a battery may still be unable to say what the scores mean for this person, at this moment, in this setting, and with what margin of error. The purpose of this book is to develop that second competence: the disciplined movement from data to a conclusion that another qualified professional could inspect, challenge and, where necessary, overturn.

Three commitments organise the chapters that follow. The first is that assessment exists to answer a question that matters. Instruments, interviews and records are means; the referral question is the end that gives them sense. The second is that every conclusion is probabilistic. Scores carry error, informants disagree, and diagnostic categories are useful simplifications of heterogeneous presentations. The third is that assessment is an ethical act. Its conclusions can open or close access to treatment, education, employment, custody and liberty, and the assessor therefore owes the client a transparent account of what was measured, what was inferred and what remains uncertain.

![FIG 1.1|The movement from referral question to responsible decision. Each step constrains the next; the final decision inherits the uncertainties recorded at every earlier step.|Referral question ; Competing hypotheses ; Methods chosen ; Evidence weighed ; Integrated decision]

Figure 1.1 presents the logic of the book in compressed form. The remaining sections of this chapter elaborate each element of that sequence, and the chapters that follow apply it to interviewing, measurement, specific instrument families and specific clinical presentations. Throughout, the emphasis falls on reasons rather than labels: a diagnosis that cannot be justified in writing is not yet a diagnosis, only an impression.

1.1 The Place of Assessment in Counselling Psychology

Counselling psychology has historically defined itself by attention to the person in context, to developmental and relational history, and to strengths as well as deficits. Assessment in this tradition is therefore not a diagnostic ritual performed before therapy begins, but a continuous and collaborative activity that informs formulation, monitors change and structures the ending of treatment. Its distinctive contribution is the integration of standardised measurement with an account of meaning: what the difficulty is, what it means to this person, and what function it serves in their life.

Assessment and testing are not synonyms

Testing is a procedure: a standardised stimulus set, a specified response format, a scoring rule and a norm or criterion against which performance is compared. Assessment is the broader process of which testing may or may not form a part. It begins with a question, selects methods proportionate to that question, integrates data from multiple sources, and ends with a communication that is useful to somebody. Meyer and colleagues (2001) demonstrated that psychological tests generally achieve respectable validity for the constructs they measure, but the same evidence shows that validity attaches to *interpretations* rather than to instruments. A test does not carry its meaning with it; the assessor supplies the meaning and is accountable for it.

This distinction has practical consequences. It explains why a technically well-administered battery can still yield a poor assessment, and why an assessment conducted without standardised instruments can, in some circumstances, be defensible. It also explains why the competence required for advanced practice cannot be reduced to test administration: the difficult skills are question formulation, method selection, integration and communication.

The institutional context of assessment

Assessment is commissioned by someone. Referrals arrive from general practitioners, psychiatrists, schools, courts, employers, insurers, adoption agencies and, increasingly, from clients themselves. Each source brings an implicit theory of what the problem is and what a useful answer would look like. A paediatric referral asking whether a child "has ADHD" has already framed the difficulty as an individual disorder rather than, say, a classroom-fit problem; a court asking about parenting capacity has framed it as a prediction about future behaviour. Part of advanced competence is recognising these frames early, negotiating them where they are inadequate, and recording the negotiation.

Counselling psychologists also work in settings where the assessment itself is an intervention. A careful history can be the first occasion on which a client's experience has been taken seriously and organised coherently; feedback can reframe a self-condemnatory narrative. Finn and Tonsager (1997) argued that assessment information can produce measurable therapeutic benefit when it is shared collaboratively, and this finding has been absorbed into what is now called therapeutic or collaborative assessment. The implication is not that assessment must always be therapeutic, but that its interpersonal dimension is never neutral.

Assessment as a professional identity

For the doctoral practitioner, assessment competence is a marker of professional standing. It signals the capacity to hold technical and relational knowledge together: to know the standard error of a difference, and also to know that a client who arrives twenty minutes late may be communicating something about the assessment. The chapters of this book are organised to develop both capacities simultaneously, because in practice they cannot be separated.

1.2 Assessment as a Decision Process

An assessment is best understood as a decision tree rather than a data-collection exercise. The first decision concerns scope: which questions are answerable within the resources available, and which must be referred elsewhere. The second concerns methods. The third concerns thresholds: how much evidence is required before a conclusion is offered. The fourth concerns communication: what the client, the referrer and any other legitimate audience need to know. Each decision narrows the space of possible conclusions, and each should be revisable in the light of new evidence.

Sequential and iterative structure

Although assessment plans are written in sequence, real assessments loop. A discrepancy between interview and questionnaire may send the assessor back to the history; a collateral report may reopen a question that had appeared settled. Explicitly acknowledging this iterative structure protects against the common error of treating the initial plan as a contract. In practice, three to five significant revisions of a hypothesis set are normal in a complex adult assessment, and records that show no revision at all usually indicate that no genuine hypothesis was ever tested.

Efficiency is an ethical requirement

Because assessment consumes the client's time, money and emotional resources, proportionate selection of methods is an ethical as well as an economic matter. Every additional instrument must be justified by the probability that it will change a decision. This principle, sometimes called incremental validity, is the strongest available argument against the indiscriminate batteries that characterise inexperienced practice. Where a battery is retained for reasons of convention or institutional requirement, the assessor should say so rather than implying that each instrument earned its place.

Documenting decisions

The decision record is the assessor's protection and the client's guarantee. A workable record states the referral question in the referrer's own terms, the questions the assessor intended to answer, the methods chosen and the reasons for them, the hypotheses under consideration, the evidence that would discriminate between them, and the point at which the evidence was judged sufficient. This record need not be long, but it must be specific enough that a colleague could reconstruct the reasoning and identify precisely where they would have decided differently.

1.3 From Referral Question to Assessment Plan

Referral questions vary in precision from the unusable ("please assess") to the over-specified ("please confirm PTSD"). The assessor's task is to convert whatever arrives into one or more answerable questions, each with a stated decision attached. "Is this person depressed?" is not answerable in the abstract; "Is the current presentation better explained by a depressive episode than by hypothyroidism, grief or burnout, and would the answer change the next treatment step?" is.

Qualities of a well-formed assessment question

A well-formed question specifies the population or person, the constructs at issue, the comparison or decision that the assessment must inform, and the standard of evidence the decision requires. It also specifies a time frame: whether the question concerns current state, recent episode or lifelong pattern determines whether state measures, retrospective interviewing or informant history will carry most weight. Where the referrer cannot supply these elements, the assessor supplies provisional ones and records them as assumptions.

Proportionality

The plan should be proportionate in three senses: in cost and burden relative to the stakes; in intrusiveness relative to the question; and in precision relative to the decision threshold. A screening question about alcohol use in a university counselling service may be answered by a validated self-report questionnaire; a question about fitness to stand trial requires a different order of evidence, different qualifications and different reporting conventions. Proportionate planning also means anticipating what will *not* be done and stating why.

Planning for what could go wrong

Competent plans include contingencies. If the client does not attend, if an interpreter is needed, if records are unavailable, if the client's reading level prevents a self-report measure, if risk emerges mid-assessment — each contingency should have a planned response. Table 1.1 sets out the planning decisions that most often determine whether an assessment succeeds, together with the failure mode that each decision prevents.

Table 1.1. Planning decisions, their rationale, and the failure they prevent

Planning decision	Why it matters	Failure prevented
Define the decision the assessment must inform	Methods are selected for their bearing on a decision, not for their prestige	Over-testing; reports that answer no question
Specify constructs before selecting instruments	Construct clarity prevents measuring convenient proxies	Score-led assessment in which the battery defines the problem
State the required standard of evidence	High-stakes decisions need converging, independent evidence	Conclusions that outrun the data
Plan the comparison or alternative explanations	Differential reasoning requires rival hypotheses to be explicit	Confirmation bias; premature closure
Plan for language, literacy and sensory access	Validity depends on comprehension of the task	Misinterpreted scores presented as ability
Set the communication plan at the outset	Feedback shapes whether findings are used	Reports that are filed but never acted upon
Record what will not be assessed	Scope limits are part of the conclusion	Implied claims beyond competence or remit

1.4 Diagnosis, Formulation and Treatment Planning

Three products emerge from clinical assessment, and confusing them is one of the most common sources of poor practice. A *diagnosis* is a classification decision: does this presentation meet the criteria for a recognised disorder, and which one? A *formulation* is an explanatory account: why this person, at this time, in this context, has developed and maintained these difficulties? A *treatment plan* is a set of actions derived from both. Diagnostic classification is retrospective, rule-governed and comparatively reliable; formulation is prospective, theory-laden and comparatively personal.

What diagnosis does and does not do

Diagnosis does several things well. It supports communication between professionals, links presentation to an evidence base, underwrites access to services and reimbursement, and imposes a discipline of explicit criteria that reduces idiosyncratic labelling. It does not explain aetiology, predict individual response to treatment reliably, or capture the person's own understanding. Nor is it stable across all presentations: mild and mixed presentations frequently fail to meet full criteria while still requiring treatment, which is why residual and unspecified categories exist and why their use should be documented rather than apologised for.

Formulation as the bridge

Formulation uses assessment evidence to construct a testable account of onset, maintenance and change. A useful formulation specifies predisposing factors, precipitants, maintaining processes, protective resources and the practical barriers that limit recovery. It should be expressed in language the client can contest, and it should predict something: if the formulation is right, then a specified intervention should produce a specified change. Where a formulation makes no predictions, it is usually a description rather than an explanation.

From formulation to plan

Treatment planning translates formulation into sequenced action, and the translation is where assessment earns its practical value. A plan that names a therapy but not the target mechanism is a plan that cannot be evaluated. Good plans specify what will be addressed first, why that target was prioritised, what would count as progress, and what would prompt a return to assessment. They also acknowledge constraints: waiting lists, cost, family obligations and the client's own preferences legitimately shape what is feasible, and a plan that ignores them is a document about an imaginary client.

1.5 Dimensional and Categorical Thinking

Psychopathology can be described categorically, as the presence or absence of a disorder, or dimensionally, as a position on one or more continuous distributions. The two descriptions answer different questions and are not in competition. Categorical description is indispensable for communication, service eligibility and legal decision-making; dimensional description preserves information about severity and subthreshold variation that categories discard.

Evidence bearing on the distinction

Structural research on the classification of mental disorders has consistently found that comorbidity is the rule rather than the exception, that symptom distributions in the population are continuous rather than bimodal, and that the boundaries between neighbouring categories are porous. The current editions of both major classifications have responded unevenly to this evidence. The *Diagnostic and Statistical Manual of Mental Disorders*, fifth edition, text revision (DSM-5-TR; American Psychiatric Association, 2022) retains a predominantly categorical architecture while adding dimensional severity ratings, cross-cutting symptom measures and a hybrid model for personality disorders in its emerging-measures section. The eleventh revision of the *International Classification of Diseases* (ICD-11) characterises personality disorder by severity of disturbance with optional trait qualifiers, a substantially dimensional departure (World Health Organization, 2024).

Consequences for the assessor

Dimensionally-minded assessors report severity and threshold information alongside any diagnosis: not only that criteria are met, but how far above threshold the presentation lies, and which criteria are marginal. They record subthreshold presentations explicitly rather than forcing them into residual categories. They treat a diagnostic boundary as a decision rule with an error rate rather than a fact about the person. And they resist the temptation to resolve genuine diagnostic uncertainty by choosing the category that seems least objectionable, a practice that is common, well-intentioned and usually misleading.

1.6 Sources of Assessment Evidence

Different methods answer different questions, and the most persistent error in advanced practice is asking one method to do the work of another. An interview yields meaning, chronology and the client's own account; systematic observation yields behaviour in context; informant report yields the perspective of someone who sees the client in settings the assessor never will; records yield documented history and prior reasoning; standardised measures yield comparison with a reference group. Table 1.2 summarises the distinctive contribution and characteristic limitation of each source.

Table 1.2. Sources of assessment evidence: contribution and characteristic limitation

Source	Principal contribution	Characteristic limitation
Clinical interview	Meaning, chronology, context, the client's account	Vulnerable to recall bias, impression management and interviewer effects
Behavioural observation	Actual behaviour in a specific setting	Reactivity; limited sampling across settings
Informant report	Perspective of family, partners, teachers or carers	Informant distress, motive and opportunity to observe
Records and prior reports	Documented history; earlier reasoning and its errors	Uneven quality; outdated conclusions recycled uncritically
Standardised tests	Normative comparison; quantified error	Construct and population specificity; administration constraints
Performance validity measures	Whether effort and engagement were sufficient	Imperfect sensitivity; not a test of honesty
Biological and medical investigations	Ruling out or identifying medical contributors	Not diagnostic of psychological disorders; access and cost
Self-monitoring and ecological measures	Behaviour in natural settings over time	Adherence; reactivity; data burden

Convergence and divergence

Multimethod assessment is valuable not because agreement is reassuring, though it is, but because disagreement is informative. A client who reports severe depressive symptoms while presenting as organised, socially engaged and functionally intact has supplied the assessor with a discrepancy that requires explanation. Possible explanations include situational fluctuation, concealment, insight differences, measurement reactivity, informant perspective or a genuine mismatch between symptom report and functional capacity. Each implies a different next step. Averaging the two sources destroys exactly the information that would have resolved the question.

Choosing among sources

Source selection follows from the question. Questions about frequency and topography of behaviour favour observation and self-monitoring; questions about developmental history favour records and informants; questions about current severity favour standardised state measures; questions about meaning favour interview. When resources permit only one source, the assessor should state in the report which claims rest on that source alone, and how confident the conclusion can therefore be.

1.7 Base Rates and Clinical Probability

One of the most consequential findings in the assessment literature is that clinicians reason poorly about probability. The base rate of a condition in the setting where the assessment occurs determines how much weight a positive finding can bear. Where a condition is rare, even a highly specific test generates more false positives than true positives, because the pool of people without the condition is so much larger. This is not a technicality; it is the arithmetic that governs screening programmes, forensic opinions and diagnostic impressions formed in specialist clinics.

The arithmetic in practice

Suppose a screening instrument with sensitivity of .90 and specificity of .90 is used in a setting where the condition affects 2 per cent of those assessed. Of 1,000 people, 20 have the condition and 18 are correctly identified; 980 do not, and 98 are incorrectly identified as positive. A positive result therefore carries a probability of roughly 16 per cent that the condition is present. In a setting where prevalence is 30 per cent, the same instrument yields a positive predictive value above 70 per cent. The instrument has not changed; the setting has. Interpreting a score without knowing the setting is therefore an arithmetic error, not merely a stylistic one.

Consequences for threshold setting

Base rates also govern where a decision threshold should sit. Where the cost of a missed case is high and the cost of a false positive is moderate — suicidal risk in an emergency department, for instance — a lower threshold is defensible, provided the assessment plan includes a way to resolve the false positives. Where a positive result triggers an irreversible consequence — a child protection removal, a fitness-for-duty determination — the threshold must be higher and the evidence converging. The assessor's responsibility is to make this trade-off explicit rather than to pretend that a cut score carries intrinsic authority.

Clinician behaviour

Research on clinical judgement has documented systematic departures from normative probability reasoning, including insensitivity to base rates, overconfidence in sparse evidence and premature closure on an early hypothesis. The remedy is structural rather than exhortatory: routine statement of the setting's base rate, routine statement of the likelihood ratio implied by each finding, and routine review by a colleague before high-stakes conclusions are issued. These habits are more reliable than the intention to be unbiased, which everyone possesses and which almost no one can sustain unaided.

1.8 Functional Impairment and Context

Diagnosis describes the pattern of symptoms; impairment describes what the pattern prevents. The two are correlated but far from identical, and the difference matters for treatment planning, for eligibility decisions and for the ethical requirement not to pathologise understandable reactions. A person may meet criteria for a depressive episode while continuing to work effectively; another may fall well short of criteria while unable to leave the house. Assessment that reports only symptom counts has measured half of what the referrer needs.

Domains of functioning

Functioning is best assessed domain by domain rather than as a global impression. Work or study, household management, financial organisation, self-care, close relationships, parenting, social participation and health behaviour each carry different demands and different consequences of failure. Within each domain, the assessor distinguishes capacity from performance: what the person could do under optimal conditions, and what they actually do in their current environment. The gap between the two is often the most clinically informative finding in the assessment.

Context as an explanatory resource

Context is not noise to be controlled but information to be interpreted. Behaviour that appears disordered in one setting may be adaptive in another: hypervigilance in a context of genuine danger, withdrawal in a context of relentless humiliation, perfectionism in a context of conditional approval. An assessor who records only the behaviour has described a symptom; an assessor who records the behaviour and the conditions that elicit and maintain it has described a person. This is the sense in which counselling psychology's attention to context is a technical asset rather than a sentimental preference.

Reporting impairment

Impairment should be reported in behavioural terms that the referrer can use: not "moderate impairment" alone, but what the person can no longer do, for how long, with what accommodation and at what cost. Where impairment is disputed — as it often is in disability, insurance and educational determinations — the assessor should describe the evidence on which the judgement rests, including the client's own account, observed performance, informant report and any documentary corroboration, and should state what was not verified.

1.9 Evidence-Based Assessment

Evidence-based practice in psychology requires the integration of the best available research evidence, clinical expertise and the client's characteristics, culture and preferences. Applied to assessment, the principle implies that instrument selection, interpretation and decision thresholds should each be justified by evidence rather than convention. An instrument earns its place by demonstrated reliability and validity for the population and purpose at hand, by incremental validity over cheaper alternatives, and by evidence that its use improves outcomes.

What the evidence base covers

The evidence relevant to assessment is broader than test manuals. It includes research on the diagnostic accuracy of instruments and interviews, on clinical judgement and its biases, on the effects of feedback on treatment engagement, and on the outcomes of assessment-driven decision-making. Assessment research has been unevenly translated into routine practice: many widely used instruments have limited independent validation in the populations where they are most often applied, and some well-validated procedures are under-used because they are unfamiliar or laborious. The assessor's obligation is to know the state of the evidence for what they use, including its gaps.

Expertise and its limits

Clinical expertise is a legitimate component of evidence-based practice, but it is not a licence for idiosyncrasy. Expertise shows itself in the quality of question formulation, in the recognition of when a standard procedure does not fit, in the calibration of confidence to evidence, and in the willingness to seek consultation. It does not show itself in the confident impression that contradicts the data. Where expertise and evidence conflict, the conflict should be recorded and, where possible, tested rather than resolved by seniority.

Client characteristics and preferences

The third element of evidence-based practice is the client. Language, culture, age, education, sensory and cognitive capacity and the client's own goals all affect whether an assessment is valid and whether its conclusions are useful. A measure validated in one population and administered to a client from another is not automatically invalid, but its validity is an empirical question that the assessor must address rather than assume. Chapter 14 treats this question in detail; here it is enough to note that cultural validity is a property of an interpretation, not a courtesy extended to a client.

1.10 Interprofessional Assessment

Complex presentations are rarely assessed by one profession alone. Psychiatrists contribute medical formulation and pharmacological reasoning; neurologists and physicians contribute differential medical explanation; speech and language therapists, occupational therapists and educational psychologists contribute functional and developmental expertise; social workers contribute safeguarding and systemic knowledge; nurses contribute observation across time. The counselling psychologist's contribution is typically the integration of psychological measurement with formulation, and the quality of that contribution depends on the quality of the exchange around it.

Communication across professional boundaries

Interprofessional assessment fails most often at the boundary rather than within each profession. Referrals arrive without the information needed to plan; reports are written in vocabulary the receiving professional cannot use; recommendations assume resources that do not exist in the receiving setting. Effective practice includes agreeing at the outset what each party will contribute, what format the output should take, and who holds responsibility for what. Where disagreements persist — about diagnosis, about risk, about capacity — the disagreement itself should be documented, because the next clinician will need to know that the question is contested.

Role boundaries and competence

Interprofessional working also requires clarity about competence. The assessment of neurological disorder, of medication effects, of legal capacity and of child protection risk each carry requirements that a doctoral qualification in counselling psychology does not by itself satisfy. Recognising the boundary of one's competence, referring appropriately and contributing what one is qualified to contribute are marks of seniority rather than admissions of inadequacy.

1.11 Limits of Assessment Conclusions

Every assessment conclusion is bounded by the methods that produced it, the population in which those methods were validated, the conditions under which they were administered and the time at which they were made. Advanced practice consists partly in stating those bounds precisely. "Within the limits of a single-session assessment using self-report measures in a client whose literacy in the test language was uncertain, the evidence is consistent with a moderate depressive episode; alcohol use was not assessed and may affect the picture" is a conclusion that can be acted upon. The same finding reported as "moderate depression" cannot.

Sources of uncertainty

Uncertainty enters at four points. Measurement error affects every score and should be expressed as a confidence interval rather than a point estimate. Sampling error affects every inference from a normative group to an individual. Method variance means that different methods measure partly different things. And genuine indeterminacy exists where the available evidence is compatible with more than one explanation. Only the last is usually acknowledged in reports; the first three are more common and more tractable.

Time and change

A single assessment describes a period, not a person. Symptoms fluctuate with sleep, substances, medication, menstrual cycle, seasonal change, relationships and circumstance. Conclusions about stable traits require evidence of stability, and conclusions about change require repeated measurement with instruments capable of detecting it. Where an assessment is used to make a decision that will persist — a diagnosis that will follow a child through school, a determination of capacity — the report should state its own expiry conditions: what change would require reassessment.

The limits of the assessor

Finally, the assessor's own limits belong in the account. Fatigue, unfamiliarity with a language or culture, a strong reaction to the client, an institutional pressure toward a particular conclusion: these are not disqualifying, but they are relevant. Consultation, supervision and, where appropriate, disclosure to the client are the available remedies. A report that records "assessment conducted without access to school records; conclusions about developmental history are provisional" is more professional than one that conceals the gap.

1.12 Integrating Findings into Responsible Decisions

Integration is the point at which the book's central skill is exercised. It requires holding several kinds of evidence in view at once, weighing them according to their quality and relevance, and producing a conclusion that is both decisive enough to guide action and honest about its margins. The common failure modes are well documented: the primacy effect of the first hypothesis, the anchoring effect of a prior diagnosis in the records, the tendency to average discrepant data rather than investigate the discrepancy, and the tendency to write reports that describe the assessment rather than answer the question.

A workable integration procedure

A defensible procedure has five steps. First, restate the decision the assessment was commissioned to inform. Second, list the hypotheses that were under consideration, including the ones that were discarded. Third, array the evidence relevant to each, noting its source, quality and direction. Fourth, state which hypothesis is best supported and why, and what evidence would have changed the conclusion. Fifth, translate the conclusion into recommendations that are specific, feasible and linked to identified needs.

This procedure has the additional advantage of being auditable. A colleague reading the record can see which evidence was weighed, which was omitted and whether the conclusion follows. Where the procedure reveals that the evidence is insufficient for any conclusion, that is itself a finding and should be reported as such, with a specification of what further assessment would resolve.

Recommendations that can be used

Recommendations are the part of the report most likely to be acted upon and most likely to be written carelessly. Effective recommendations name an action, an agent, a setting and a review point: "the school to provide extended time in written examinations for the next two terms, reviewed at the end of the spring term with a re-administration of the timed reading measure" is usable; "support with examinations" is not. Recommendations should be proportionate to the evidence, should distinguish what is required from what is desirable, and should not smuggle in clinical claims that the assessment did not establish.

Closing the loop

Assessment ends with feedback and follow-up, not with the signature on the report. Feedback gives the client an opportunity to correct factual errors, to contest interpretations and to influence the plan; follow-up establishes whether the recommendations were implementable and whether the formulation predicted what happened. Both practices improve the accuracy of subsequent assessments, because they expose the assessor to the consequences of their inferences. The ethical warrant for assessment is that it helps; the empirical test of that warrant is whether it did.

Advanced Case Conference

The referral. A community mental health team asks for assessment of a 34-year-old man, referred by his general practitioner, with the question: "Does he have bipolar disorder?" The general practitioner notes two episodes of elevated mood in five years, each following a period of heavy alcohol use and each lasting four to six days; a long history of unstable employment; and a recent presentation with low mood after a relationship ended. The team's psychiatrist has already prescribed a mood stabiliser.

The evidence. Semi-structured interview establishes no episode of elevated mood lasting more than four days, no marked impairment during the episodes, and no hospitalisation. Collateral report from a sibling describes the same two episodes as "him being on form for a week", with no change in functioning. A structured diagnostic interview supports a hypomanic episode; neither episode meets criteria for mania. Screening identifies harmful alcohol use over eight years and a current moderate depressive episode. Records document two earlier diagnoses of recurrent depression, made without assessment of alcohol use.

The reasoning. Three hypotheses compete: bipolar II disorder; alcohol-related mood instability with a superimposed depressive disorder; and a primary depressive disorder with substance use. The structured interview supports the first, but the temporal association with drinking and the absence of inter-episode elevation weaken it. The base rate of bipolar disorder in a community mental health setting is low enough that a single supporting source should not carry the conclusion. Discriminating evidence would be prospective mood charting over three months, corroborated by an informant, together with a period of reduced drinking.

The decision. The report records a provisional diagnosis of bipolar II disorder with a stated confidence level, documents harmful alcohol use as a co-occurring focus of treatment, and recommends prospective monitoring before any change to the mood stabiliser is considered. It states explicitly what would change the conclusion: a third episode independent of substance use, or prospective charting showing sustained elevation with functional change. The general practitioner is advised that the diagnostic question remains open, and that the current treatment plan does not depend on its resolution.

The teaching point. The assessment did not answer the question that was asked, and saying so was the correct outcome. A conclusion that answered it would have required evidence the assessment did not have.

Chapter Summary

Assessment in counselling psychology is a decision process organised around a question, not a battery organised around an instrument. Its products are distinct: diagnosis classifies, formulation explains and treatment planning prescribes, and conflating them produces reports that neither classify accurately nor explain usefully. Interpretation is probabilistic throughout, and base rates, measurement error and method variance determine how much a finding can bear. Functional impairment and context are not ancillary information but part of what a diagnosis means for a particular person. Evidence-based assessment requires instrument selection and threshold setting to be justified by evidence for the specific purpose at hand, and it treats clinical expertise as a discipline of calibration rather than a licence for impression. Finally, the limits of an assessment are part of its conclusion: a report that states its own uncertainty is more useful, and more honest, than one that does not.

Review Questions

1. Distinguish testing from assessment, and explain why the distinction matters for interpreting a score.
2. Using an example from your own placement or service, state a referral question in the form specified in Section 1.3 and identify the decision it informs.
3. Calculate the positive predictive value of a test with sensitivity .85 and specificity .80 in settings with prevalence of 5 per cent and 40 per cent. What follows for screening policy?
4. Explain why a discrepancy between self-report and informant report should be investigated rather than averaged.
5. What does it mean to say that the limits of an assessment are part of its conclusion? Illustrate with a report sentence.

Discussion Questions

1. A court asks you to determine whether a parent has a personality disorder. How do you negotiate the referral question, and what do you record about the negotiation?
2. The team psychiatrist has already started treatment. What are the professional and ethical considerations in reporting a provisional rather than a confirmed diagnosis?
3. Should a diagnosis ever be withheld from a client at the request of a third party? Argue both positions and state your own with reasons.
4. How would you decide whether a new instrument has earned a place in your service's assessment protocol?
5. In what circumstances, if any, is an assessment without standardised instruments defensible?

Further Reading

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